Weekly cetuximab + FOLFOX-4

NCCP 00692 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Visits, access and care between treatments

Cetuximab on Day 1 and Day 8. FOLFOX4 starts on Day 1: oxaliplatin, folinic acid and fluorouracil bolus, followed by a 22-hour fluorouracil infusion. Day 2 includes folinic acid, another fluorouracil bolus and a second 22-hour infusion; finish around Day 3. Repeat every 14 days while beneficial and tolerable.

STEP 1

Before the first visit

Confirm the exact regimen, tests, medicines, emergency number and treatment goal. Discuss which central line is suitable and arrange pump training.

STEP 2

At the treatment unit

The first weekly cetuximab infusion takes about two hours; later weekly infusions usually take about one hour. Oxaliplatin and folinic acid are usually infused over about two hours, before fluorouracil. Blood review, preparation, premedicines and observation add time; infusion time is not the total appointment length.

STEP 3

After the infusion

Return on Day 2 for the second folinic-acid/fluorouracil part and renewal of the second 22-hour infusion. Final disconnection is planned around Day 3. This FOLFOX4 plan is not a single 46-hour infusion.

STEP 4

Between doses

Attend the separate Day 8 cetuximab-only visit. This is not another irinotecan or oxaliplatin day. Use the written symptom and support plan; do not wait for the next appointment if urgent symptoms appear.

STEP 5

The next decision

Confirm the next visit after any delay or dose change. Treatment continues only while the oncology team judges benefit and tolerability acceptable. Follow-up mineral checks continue after the final EGFR-antibody dose where applicable.

Transport and daily life

Plan transport if antihistamines, symptoms or other medicines affect driving. Ask for practical, work, nutrition or emotional support.

If treatment changes

Ask which component has been held and which visits remain. A treatment delay does not automatically cancel blood tests, line care or symptom follow-up.

Sources: NCCP 00692 v2 · official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.